

Letter to the Editor

Invisible wounds, visible crisis: rethinking chronic pain, PTSD, and traumatic brain injury in veterans worldwide

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TO THE EDITOR

I read with great interest the recent article by Liu et al. [1], titled “Knowledge, mechanisms, and intervention of the polytrauma clinical triad in military pain medicine,” which provides a comprehensive overview of the pathophysiology, interrelated mechanisms, and therapeutic approaches to managing the Polytrauma Clinical Triad, chronic pain, PTSD, and traumatic brain injury (TBI), in military populations. The authors highlight integrative treatment strategies that include multimodal pain management, neuropsychological interventions, and rehabilitative care tailored to the overlapping nature of these conditions. Their timely insights underscore the urgent need to move beyond siloed care, recognizing this triad as a deeply interwoven clinical challenge. The cries of pain they describe echo far beyond the battlefield and demand global attention.

Today, pain is no longer just a battlefield injury, it is the aftershock that follows soldiers home [2]. From Seoul to São Paulo, veterans face an invisible war of a different kind: pain that lingers long after the guns fall silent, trauma that resurfaces in dreams, and brain injuries that quietly corrode memory, balance, and identity [3]. This

triad, PCT, is not only a clinical challenge, but a moral one. How we respond reveals not just the strength of our science, but the depth of our compassion.

The path forward must be integrative and international. The traditional silos of care are insufficient. Pain specialists, neurologists, mental health professionals, and rehabilitative teams must work as one, guided by a biopsychosocial approach that treats the whole person, not just their symptoms [4]. In this model, individualized care plans, shared decision-making, and culturally sensitive treatment become essential tools in recovery.

Equally vital is the dismantling of stigma. PTSD and TBI, while often invisible, are no less real than shrapnel wounds. In many nations, including those with advanced medical systems, silence remains a barrier to diagnosis and care [3]. Soldiers must be empowered to speak, and systems must be equipped to listen—early, empathetically, and effectively.

South Korea, with its sophisticated medical infrastructure and lived history of conflict, is uniquely positioned to lead this global movement. Its advances in digital health, wearable diagnostics, and neurorehabilitation can shape the future of PCT care, not only regionally but worldwide. Collaboration across borders, scientific, military, and hu-



manitarian, is no longer optional. It is a necessity.

The battlefield has changed. The wounds have evolved. So must our medicine.

Let us meet this triad not with despair, but with determination, to heal not only the broken body, but the fractured soul.

DATA AVAILABILITY

Data sharing is not applicable to this article as no datasets were generated or analyzed for this paper.

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CONFLICT OF INTEREST

No potential conflict of interest relevant to this article was reported.

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AUTHOR CONTRIBUTIONS

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